

CONSULTATION & COLLABORATION

Peer Consultation Group Charter

Everything a new peer consultation group needs on day one: purpose, ground rules, confidentiality, AI and recording rules, agenda, log and health check.

AUDIENCE	FORMAT	VERSION	TIME TO ADAPT
Psychologists · Psychiatrists	Charter + agenda + log	1.0 · Sept 2026	30 minutes as a group

WHAT IT IS	A one-document charter for a closed peer consultation group of four to eight clinicians, with a member agreement, meeting agenda, consultation log and quarterly health check.
USE IT WHEN	You are forming a group, or an existing group has never written down its rules and wants to.
CUSTOMIZE FIRST	Meeting cadence and format, how members present cases, the group's written stance on recording and AI note-takers, and how you handle a concern about a member.

Why a charter

Consultation is one of the best protections against clinical error and professional isolation, and a group contract is what makes it safe to bring real cases. Peer consultation is not supervision: the treating clinician keeps full responsibility for the patient, and the group offers perspective, not orders. A good charter defines confidentiality, attendance and feedback norms before the first hard case arrives.

	Peer consultation	Clinical supervision
Relationship	Equals; shared responsibility for the group	Supervisor has authority and evaluative role
Responsibility for the patient	Stays with the treating clinician	Shared, and often legally with the supervisor
Purpose	Perspective, ethics, blind spots, support	Training, oversight, licensure hours
Records	Consultation log; no patient records shared	Supervision log; may include record review
Document to use	This charter	Supervision Agreement

Part A: Charter

1. Purpose

[Group name] exists to give members a confidential space to consult on clinical, ethical and professional questions, reduce isolation, and improve the quality of patient care. Consultation offers perspective. It does not replace supervision, legal advice or a member's own clinical judgment.

2. Membership and format

- Closed membership of [4 to 8] licensed clinicians. New members join by consensus after attending [one] meeting.
- Meets [weekly / every two weeks / monthly] for [60 to 90] minutes, [in person / by HIPAA-compliant video / hybrid].
- Trial period of [3 to 4] months, then a group review of what to keep and change.
- Rotating roles each meeting: facilitator, timekeeper and note-taker (for the group's own log only).

3. Confidentiality and de-identification

APA Ethics Code Standard 4.06 allows consultation without identifying information, or with the patient's consent, and only to the extent necessary for the purpose. The group adopts these rules:

- Cases are presented without names, initials, employers, unusual places, dates of birth or other details that could let a member recognize the patient. Use the De-identification Checklist in the consultation template.
- Any member may stop a presentation if they think a patient may be recognizable, including a patient the member knows. Presenters do not name the patient's community if it is small.
- A member who realizes they know or treat someone being discussed says so at once and leaves the discussion of that case.
- Patients' informed consent notes that the clinician consults with peers without identifying information (see the Informed Consent template).
- What is said about members' own work, mistakes and personal reactions stays in the group.

4. Recording, notes and AI tools

- No audio or video recording of meetings.
- No AI note-takers, transcription or summarization tools in any meeting, in-person or online, including those built into video platforms.
- Members may take brief written notes for their own records of what they were told and what they decided. A presenter may write a short summary of the feedback received, ideally within 24 hours, for their own consultation log.
- No patient information is posted in chat, shared documents or messaging apps.

5. Feedback norms

- The presenter states the consultation question first.
- Ask before advising: "Do you want ideas, reflection or a sounding board?"
- Curious and specific, not judgmental. Name what is working before naming what is not.
- Consensus is not required. The presenter decides and owns the decision.

6. Attendance and commitment

Members attend at least [80] % of meetings, let the group know in advance if they cannot, and come prepared. Two consecutive absences trigger a check-in about fit.

7. Conflicts and dual relationships

Members disclose personal or professional relationships that could compromise objectivity (shared patients, supervisory, family, business). Members who share a patient do not present that case in the group.

8. Concerns about a member's practice

If a member has a concern about another member's competence, impairment or conduct, they first raise it directly and privately. If it involves risk to patients or an ethics standard that requires action, the member seeks separate consultation and follows their reporting obligations. The group is not a place to hide conduct that requires reporting.

9. Documentation

Presenters record in their own consultation log the date, that consultation was sought, the question, the main perspectives offered and their decision. Consultation is documented in the log rather than in a patient's chart unless your policies or payers require otherwise.

10. Leaving, ending and review

Any member may leave with [two weeks'] notice and a final check-in. The group reviews this Charter every [six months]. The group ends by agreement, and members securely destroy any group materials.

Part B: Member agreement

I have read the [Group name] Charter and agree to follow it. I will protect the confidentiality of other members and of patients discussed, present cases without identifying information, keep no recordings and use no AI note-taking tools in meetings, and take responsibility for my own clinical decisions.

Member name

Signature

Date

Member name

Signature

Date

Member name

Signature

Date

Member name

Signature

Date

Part C: 90-minute meeting agenda

Time	Item	Notes
0–10 min	Check-in and confidentiality reminder	One line each. Facilitator reads the reminder aloud.
10–15 min	Agree the schedule	Choose presenter(s) and time boxes.
15–35 min	Case one	Presenter 5 to 7 min (Case Consultation Template), clarifying questions 5 min, discussion 8 to 10 min.
35–55 min	Case two or an ethics question	Same structure.
55–75 min	Topic: article, ethical dilemma, practice issue	Rotate who leads.
75–85 min	Presenters summarize takeaways and decisions	Each presenter states what they will do next.
85–90 min	Closing round and next meeting details	Note-taker records attendance and next presenters.

Part D: Consultation log (for the presenter's own records)

Date	Group	Patient ID (your own code, not initials)
Consultation question		
Main perspectives offered		
My decision and rationale		
Follow-up and date	Signed	

Part E: Quarterly health check

Each member answers privately, then the group discusses themes.

- ☐ Do I feel safe bringing a difficult or embarrassing case here?
- ☐ Are meetings starting and ending on time, with fair airtime?
- ☐ Is our de-identification good enough that I would be comfortable if my patient heard the presentation?
- ☐ Are we getting the mix of cases, ethics and practice topics we need?
- ☐ Is anyone dominating or silent?
- ☐ What would make this group more useful next quarter?

IN PSYALLIANCE

Use Town Hall specialism channels and the Network to find members with complementary specialisms. Keep your group's charter in your personal library, and never post case material in shared spaces.

Sources and further reading

- APA Ethics Code, Standard 4.06 Consultations and Standard 4.02 ([apa.org/ethics/code](https://www.apa.org/ethics/code)).
- North Carolina Psychological Association, Peer Consultation Group resources (ncpsychology.org).
- Zur Institute, "The Role of Peer Consultation Groups in Maintaining Clinical Resilience" (2026).

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

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